



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متدد

إسم المريض ربيعاً : رانده صلاح الدين عباس ابراهيم

المحافظة : الجيزة

رقم البطاقة : 27601260104441 رقم التليفون : 01002244845

العمر (تاريخ الميلاد) : 26-01-1976 المهنة : ربه منزل

العنوان : الهرم

تاريخ تسجيل الإستمارة : 04-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

المقر بما فيه
التوقيع: (.....)

الإحصائي الاجتماعي
التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام تصوير مقطعي محوسب للصدر بالصبغة [10851] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني	الوظيفة:	التوقيع:
الاسم: د/ندى حسن	الوظيفة:	التوقيع:
الاسم: د/نرمين رمضان	الوظيفة:	التوقيع:

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: رانده صلاح الدين عباس ابراهيم
National ID: 27601260104441



تقرير طبي

المسجد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- -- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Contrast-enhanced CT scan of the chest.

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين



٢٠٢٠/٠١ ٢٧٦.١٢٦.٠١.٤٤٤١
حاصلة على بطوروس خدمة اجتماعية

التي مقررة

عبدالرحمن السيد احمد محمد

البطانة مصرية حتى ٢٠٢٧/٠٩/٣٠



جمهورية مصر العربية
بطانة تحقيق الشخصية

الاسم

صلاح الدين صليح ابراهيم

٢٤ زايد كومينكس عفرى زايد

الشيخ زايد - الجيزة



II2387101

٢٧٦.٠١.٢٦.٠١.٤٤٤١



Randa salah eldin abaas

24/10/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Reported case of breast carcinoma

1. Chest:

- Right breast skin thickening as well as metallic clips.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.
- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Reported case of breast carcinoma.***
- ***No definite pulmonary, hepatic or osseous metastatic deposits.***

Dr. Mohammed Mohsen, MSc.

Dr. Amr Farouk, M.D.



S C A N

Name: Randa Salah Al Din Abbas
Referred by: Prof.

ID: 3246102
Date: 18/09/2022

Thanks for reference

Bilateral Digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

Right breast :

- Normal fibroglandular architecture (ACR 1).
- Right breast UOQ dense lesion of irregular spiculated outline in the location , seen in ultrasonography as solid hypoechoic infiltrative mass mm in diameters .Associated architecture distortion .
- Normal skin thickness & contour .
- Small sized likely non-specific axillary lymph nodes.

Left breast :

- Normal fibroglandular architecture (ACR 1).
- No speculated masses or pathological microcalcification.
- Normal skin thickness & contour .
- No breast solid or cystic masses. Normal tissue elastogram.
- No architecture distortion.
- Small sized likely non-specific axillary lymph nodes.

OPINION :

Right breast UOQ malignant mass , BIRADS 5, biopsy was done.
Essentially normal study of the left breast , BIRADS 1.

Kindly accept my best regards
Dr. Naglaa Abdel Razek

Defining BIRADS classification of breast lesions according to the American College of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
BIRADS 1 : Normal study.	0%
BIRADS 2 : Benign findings.	0%
BIRADS 3 : Probably benign.	2%
BIRADS 4 : Probably malignant.	50%
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	



MDT FORM

24/11/2022 GC

Name	أنا عبد الله عبد الله	MRN	C. 99 / 20
Age	48	ID	207.157.1
Menopausal status	pre	Phone	010 22 22 22
NRS SCORE		marital status	M.
BMI		Family history	
Co morbidity	H.T.	REASON OF MDT	

Clinical Data

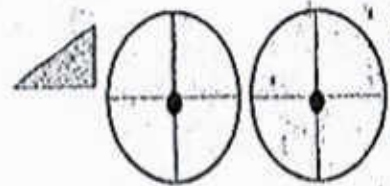
Bx: IDC

ER: 7/8 PR: 0/8 HER2: ++ CISH: + KI67: 30%

Sonomammography:

Mets Workup: CXR, Abd 4/5: pre

R

pT₁N₀M₀

BCS + SW. done 2

Recommendations:

pre - CT CAP.

+ Bone scan

+ Revise IHC here

+ Echo, lab.

+ port. a cell

MDT OUTCOME & PLAN

if same biopsy:

for ady: Tamoxifen x 12 wks + Herceptin
x 6 months

then PORT

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	ps)
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Name : Randa Salah al-Din Abbas Ibrahim

Date : 05-01-2026 10:26

Branch : Dokki

Medical No. : 268911

Ref.Physician Prof : Goustav Roussey Egypt

Ins.Company : Walkin

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after right conservative surgery.

- **Breast density:** Predominant fatty tissue ... *Matching ACR A .*
- Evidence of surgical clips in the right UOQ .
- No detected definite suspicious masses, architectural distortion or pathological calcification in both breasts .
- Normal skin and nipple in both breasts.

N.B: evidence of cassette artifact (appeared as grouped microcalcifications).

By complementary bilateral breast ultrasound:

- Heterogeneous (predominantly fatty) parenchymal echo-texture.
- Clear operative bed .No suspicious masses .
- No detected suspicious solid or cystic masses in both breasts..
- No detected significant axillary lymph nodes.

CONCLUSION:

- *Post conservative surgery changes of the right breast (BIRADS 2 Category).*
- *Normal left breast (BIRADS 1 Category).*

Much Obliged

DR.MONA ZAKY.MD

University

Professor of diagnostic radiology-Breast imaging unit -Mansoura

Dr. Hisham / Echo report

Subject:

Date: 27/2/2025

EF	54	55-75%	PWT	1.1	0.6-1.1cm
LVED	4.3	3.5-5.8cm	AOR	2.7	2-3.7cm
LVES	2.4	2-4cm	LA	3.8	2-4cm
FS	31	25-45%	TR	ref	0.7-2.3cm (supine)
SWT	10	0.6-1.1cm	ACDUSP	ref	1.5-2.6cm

LV dimensions

ref

LV wall thickness

ref

LV diastolic function

LEDD

SWMA

ref

LA and Aortic root dimensions

ref

Mitral Valve

ref i trivial MR

Aortic valve

ref

Tricuspid valve

ref i trivial TR, RVIPV

Pulmonary valve

ref

Right side and systolic function

ref

Intracardiac masses and effusion

ref

Dr. Mohammed Anwar

Asc cardiology

ref



Patient Name: Randa Salah Al Din Abbas Ibrahim	Date : 07/04/2025
Exam : ultrasound TRANS VAGINAL	Age : 49 Year(s)
Medical ID: 4094/2022	Pat. ID: 33125

Trans-vaginal ultrasound scanning revealed: -

Uterus: -

- ③ AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
- ③ The myometrium shows anterior wall intramural fibroid measuring 10x8mm (FIGO 4) with suspected other one at the fundus
- ③ The endometrial line is 9mm showing tiny cystic areas for follow up
- ③ Nabothian cysts at the cervix, the largest is 12mm

Ovaries: -

- ③ Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Conclusion:

Uterine fibroid
Mildly thickened endometrium for follow up

Signature Dr . Nermien Ramadan
07/04/2025 2:29 PM



Echo report

Name:

Yusef Yusef

Age:

4.94
cc

Date: *4/7* 2025

EF	<i>67.1</i>	54-74%	PWT	<i>0.7</i>	0.6-1.1cm
LVEDD	<i>4.3</i>	3.5-5.7cm	AO	<i>2.1</i>	2.0-3.7cm
LVESD		2.2-4.0cm	LA	<i>3.2</i>	1.9-4.0cm
FS		25-45%	RV		0.7-2.3cm (supine)
SWT	<i>0.8</i>	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV dimensions:

normal

LV wall thickness:

normal

LV diastolic function: *DD grade II*

SWMA.

0.83

LA size

normal

Aortic root dimension

normal

Mitral Valve

normal

Aortic valve

normal

Tricuspid valve

7.1d R

Resp 28mmHg

Pulmonary valve

normal

Right side size and systolic function

normal

Intracardiac masses and thrombi

normal

Pericardium

normal

IVC diameter and collapsibility.

normal

Conclusion:

D/

Yusef Yusef

Patient Name: Randa Salah Al Din Abbas Ibrahim	Date : 08/07/2024
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID : 4094/2022	Pat ID: 33125

BI-LATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION ULTRASOUND

Indications:

Right breast conserving therapy

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Predominantly fatty parenchyma (ACR a)
- Right breast:

Right breast:

- Operative bed parenchymal distortion and scarring marked by metallic clips with no sizable mass. Related hyperchole lesions are related to the scar site palpable lumpiness mostly due to fat necrosis.
- Regression of the right breast UOQ area of asymmetrical parenchymal densities.
- No suspicious mass lesions, micro calcifications
- No sizable collection
- Diffuse edema pattern in the form of increased trabeculations and skin thickness.
- No regional lymphadenopathy

Left Breast:

- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Normal retro-areola ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Post right BCS status showing no evidence of recurrence (BIRADS 2)
- Normal study of the left breast (BIRADS 1)

Signature Dr. Mohammed Mohsen



مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

الاستراتيجية
مصر
صحة

MDT FORM

6/2/2023 G C

Name	رائد محمد مصطفى			MRN	٤٠٩٤-٤٠٤٤
Age	46 yrs			ID	٤٧٧٠١٤٦٠١٠٣٤١
Menopausal status	pre menopause			Phone	٠١٠٠٥٤٤٠٩٩٩
NRS SCORE				Marital status	١١٠٠٤٤٤٨٤٠
BMI				Family history	
CO morbidity	HIND			REASON OF MDT	
Clinical Data					
BX:	201/43			R	L
ER:	PR:	HER 2:	Ki67:		
8/9	6/8	Amplified	38%		
Sonomammography:					
Mets Workup:					
- CAP CT scan → Free - Bone scan → Free					
Recommendations:					
The patient underwent upfront RT BCS + SNL → T1N0 Tumor x12 - Weight: x3 ended on 6/2/2023 - will start finished PORT					
MDT OUTCOME & PLAN					
Decision: start the treatment for Tamoxifen x12 the switch keep on adjuvant therapy					
SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
فادي	محمد	أحمد			

Presidential Initiative for Women's Health - Pathology Lab
المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Patient Name	: الأستاذة / راندا صلاح الدين عيسى إبراهيم	Request No.	: 13003141
Patient No.	: 27601260104441	Request Date	: 24/10/2022
Path No.	: SR-814/22	Verification Date	: 12/11/2022
Sex	: Female	Financial Category	: نفقة الدولة - ورام دار السلام
Age	: 46 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

CLINICAL DATA : Right breast cancer
NATURE OF SPECIMEN : Revision of Tru cut biopsy slides , IHC slides , WLE&SLN slides
GROSS EXAMINATION : 1 block (5239/22) + 12 slides
MICROSCOPIC EXAMINATION
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+mitosis) : (3+2+2) Grade II
* DCIS: Focal
* Angioinvasion: No
* Lymph node involvement: Free sentinel LNs (0/5)
* Resected margins: Free
* IPT :
* ER: Strong nuclear reaction in 75% of tumor cells (Allred score 8/8)
* PR: No nuclear reaction of tumor cells (Allred score 0/8)
* HER2: Weak complete membrane reaction of >10% of tumor cells (score 2+)
* SISH: Amplified with clusters
(Average Her2 copy No. > 6 , Her2/CEP17 ratio > 2)
DIAGNOSIS
Right breast cancer , Slides revision:
Invasive duct carcinoma, Grade II .
* Free received margins
* Free SLN (0/5) (Stage pN0)
IHC :
* ER: Positive (score 8/8).
* PR: Negative (score 0/8).
* HER2: Equivocal (score 2+).
* SISH: Amplified with clusters.

Prof. Dr. Amal Hareedy
Dr. Samah Sayed

Verified by :
saamah sayed

Printing Date : 12/11/2022 13:32:11

Page 1 of 2

Printing By : dr Enas



19911
@alborscan.com

01062730134 - 01019911595



Visit Number: 73922506427

Patient Name: د. ياسر صلاح الدين

Age / Sex: 47 year / Female

Patient ID: 2129692432

Client Name: Exclusive Card

Referred by Prof:

16 Nov 2022

Exam Date:

11/19/2022 5:55:50 PM

Printed:

NM

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

HISTORY

A 46 female patient presented with history of right breast cancer and referred for metastatic workup.

TECHNIQUE

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

FINDINGS

- No evidence of active or cold osseous tracer uptake abnormality that may account for metastatic bone disease could be detected.
- Degenerative changes are noticed at different levels of the spine and around big joints.

IMPRESSION

- The skeleton is free from metastases.

Dr. Yasser Mohamed, MD

Prof of NM Kasr Elaini Fellow
to Imperial College U.K

Prof: Yasser Mohamed M.D
Consultant N.M
Fellow Imperial College U.K.



00 ط. عبد المنعم ياسر - 04 الطباء - المهندسين

04 ط. راج شبرا

07 ط. راج المومني

09 ط. خالد الحلو - فيصل

05 ط. ياسر طارق - 85 - الدور الثالث



Randa salah eldin abaas

24/10/2022

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED:-**

Reported case of breast carcinoma

1. Chest:

- Right breast skin thickening as well as metallic clips.
- No sizable pulmonary nodules.
- No pathological hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.

- Average cardiac size. No pericardial effusion detected.

- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is average in size with homogenous CT texture. No definite focal lesions or contour irregularities.

- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.

- No intra-hepatic or extra-hepatic biliary radicles dilatation.

- Normal size & parenchymal texture of the spleen with no focal lesions seen.

- No pathological abdominal or pelvic lymph nodes.

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.

- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.

- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.

- Normal appearance of the other pelvic organs.

- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton shows no definite worrisome lytic or sclerotic lesions.

- No vertebral structural collapse or extra-osseous soft tissue component.

OPINION:

- ***Reported case of breast carcinoma.***
- ***No definite pulmonary, hepatic or osseous metastatic deposits.***

Dr.Mohammed Mohsen, MSc.

Dr. ~~Amr Farouk~~ M.D.



S C A N

Name: *Randa Salah Al Din Abbas*
Referred by: *Prof.*

ID: 3246102
Date: 18/09/2022

Thanks for reference

Bilateral Digital soft tissue mammography & high resolution breast sonography & Elastography revealed :

Right breast :

- Normal fibroglandular architecture (ACR 1).
- Right breast UOQ dense lesion of irregular spiculated outline in the 10:00 location , seen in ultrasonography as solid hypoechoic infiltrative mass 12x9 mm in diameters .Associated architecture distortion .
- Normal skin thickness & contour .
- Small sized likely non-specific axillary lymph nodes.

Left breast :

- Normal fibroglandular architecture (ACR 1).
- No speculated masses or pathological microcalcification.
- Normal skin thickness & contour .
- No breast solid or cystic masses.Normal tissue elastogram.
- No architecture distortion.
- Small sized likely non-specific axillary lymph nodes.

OPINION :

Right breast UOQ malignant mass , **BIRADS 5, biopsy was done.**
Essentially normal study of the left breast , **BIRADS 1.**

Kindly accept my best regards,
Dr. Naglaa AbdelRazek, M.D.

Defining BIRADS classification of breast lesions according to the American College of Radiology:

Category	Suspected Cancer Risk
BIRADS 0 : Non-informative study.	
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BIRADS 2 : Benign findings.	0 %
BIRADS 3 : Probably benign.	2 %
BIRADS 4 : Probably malignant	50 %
A : mild suspicion.	
B : moderate suspicion.	
C : high suspicion.	
BIRADS 5 : Malignant.	98-100%
BIRADS 6 : Pathologically proven malignant lesion.	

د/ محمد شريف

استشارى الباثولوجى

استاذ بكلية الطب جامعة القاهرة (م)

د/ أحمد زعيم

استشارى الباثولوجى

استاذ بكلية الطب جامعة القاهرة (م)

د/ محمد عبد المنعم

استشارى الباثولوجى

استاذ بكلية الطب جامعة القاهرة (م)

Name	رانيا صلاح الدين عباس	Age	46Y	Code	2061 - 22
Referred by	أد عبد خلاق	Date	1/10 /2022	Sex	Female
Clinical data	Right breast mass				
Specimen	Conservative breast surgery and sentinel lymph nodes sampling				

PATHOLOGY REPORT

GROSS: A fibrofatty tissue piece measures 8x5x4 cm, covered by a skin ellipse 6x1.5 cm. Serial sections revealed an ill defined mass measured 1.4x0.9 located 2 cm, 2.5 cm, 1 cm, 2.5 cm & 3 cm away from the following margins respectively; superior, inferior, medial, lateral, & deep. Also received 5 blue stained lymph nodes measured up to 1.5 cm in diameter.

MICROSCOPIC PICTURE: Sections examined revealed infiltrating tumor tissue formed of solid groups and sheets of malignant ductal epithelial cells exhibiting moderate grade of anaplasia and mitotic activity. The stroma was desmoplastic. In situ duct carcinoma was inconspicuous. Lymphovascular emboli and perineural invasion were not seen. Sections examined from all resection margins were free from tumor infiltration. Sections examined from the received sentinel lymph nodes showed reactive changes, free of metastatic deposits (0/5).

DIAGNOSIS: Right Breast mass, Conservative Surgery & SLN:

- > Infiltrating Duct Carcinoma (NST), Grade II.
- > Free And Adequate Resection Margins.
- > Free Sentinel Lymph Nodes (0/5).
- > Stage: pT1, N0.

Dr. Somia Soliman, MD

Dr. Ahmed Naeem, MD

Dr. Mohamed Sherif, MD

Sonia A. Soliman

A. Naeem

M. Sherif

برج النورثة فيصل - شارع فيصل الرئيسى - أمام كلية التربية الرياضية - الدور الثاني

Faisal Pearl Tower in front of the Faculty of Physical Education - Second floor

01142299928 - 0235936015

0112244444 - 02266444

Dr.

NADIA M. MOKHTAR

*Professor of Pathology
Cancer Institute, Cairo University*

دكتورة

نادية محمود مختار

أستاذ الباثولوجي
معهد الأورام - جامعة القاهرة

Name: رندا صلاح الدين عباس

Age: 46

Clinical: RT BREAST MASS

Nature: CORE BIOPSY

DR: د. خديجة عبدالقادر

5239 / 22

Pathology Report

Gross: Six cores, 1-0.5 cm each, all submitted.

Micro: All cores are positive for invasive groups of duct carcinoma cells. They show moderate anaplasia and tubule formation. Stroma is sclerotic. ER, PR, Her-2 and Ki-67 immunophenotyping will be done.

Diagnosis: RT BREAST, CORE BIOPSY: INVASIVE DUCT CARCINOMA, GRADE II.

21-Sep-2022

Prof. Nadia Mokhtar



Dr.

NADIA M. MOKHTAR

Professor of Pathology
Cancer Institute, Cairo University

دكتورة

نادية محمود مختار

أستاذة الباثولوجى
معهد الأورام، جامعة القاهرة

Name: رندا صلاح الدين عباس

Age: 46

Clinical: RT BREAST MASS

Nature: CORE BIOPSY FOR IMMUNOPHENOTYPING

DR: د. خديجة عبدالقادر

5239 / 22

Pathology Report

Gross: Six cores, 1-0.5 cm each, all submitted.

Immunophenotyping: Sections are stained using ER, PR, Her-2 and Ki-67 antibodies with Ventana Gx Immunostainer.

Micro: Immunophenotyping:

All tumor cells are positive for nuclear staining of ER with moderate intensity.

All tumor cells are negative for nuclear staining of PR.

More than 10% of invasive tumor cells are positive for complete thin membranous

staining of Her-2. No thick membranous staining, thus inconclusive, for SISH testing.

Ki-67=30% denoting high proliferation index.

Diagnosis:

RT BREAST, CORE BIOPSY FOR IMMUNOPHENOTYPING: ER STRONGLY POSITIVE 7/8,
PR NEGATIVE 0/8, HER-2 INCONCLUSIVE (SCORE +2) FOR SISH TESTING, Ki-67=30%
(HIGH).

23-Sep-2022

Prof. Nadia Mokhtar



Dr.

NADIA M. MOKHTAR

Professor of Pathology
Cancer Institute, Cairo University

دكتورة

نادية محمود مختار

أستاذ الباثولوجي
معهد الأورام، جامعة القاهرة

Name: رندا صلاح الدين عباس

Age: 46

Clinical : RIGHT BREAST CANCER

Nature: PARAFFIN BLOCK FOR HER2 DUAL ISH

DR: أ.د/ خديجة عبد القادر

5239 / 22

Pathology Report

Gross:

Paraffin block labelled 5239 of invasive tumor components is serially cut at 4 microns. VENTANA INFORM HER2 Dual ISH DNA Probe Cocktail Assay: HER2 gene status is reported as a function of the ratio of the average number of HER2 gene copies (BLACK) to the average number of Chr17 copies (RED) in nuclei of 20 interphase cells within an invasive breast or gastric carcinoma. HER2 gene status is classified as non-amplified HER2/Chr17 ratio < 2.0 or amplified HER2/Chr17 ratio ≥ 2.0 .

Micro:

HER2 gene status is :
Amplified HER2/Chr17 ratio > 6.0 with clusters.

Diagnosis:

BREAST, VENTANA DUAL ISH DNA: AMPLIFIED HER2/Chr17 ratio > 6.0 WITH CLUSTERS.

10-Oct-2022

Prof. Nadia Mokhtar





Patient Name: Randa Salah Al Din Abbas Ibrahim	Date : 08/07/2024
Exam : ultrasound ABDOMEN& PELVIS	Age : 48 Year(s)
Medical ID: 4194/2022	Pat. ID: 33125

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver:-

- average in size (14cm at Rt MCL) homogenous parenchymal echopattern with smooth surface.
- No hepatic focal lesions or intra hepatic biliary radicle dilatation.
- Portal vein is patent and not dilated.

Gall Bladder:-

- Average distention and capacity with no stones or mud
- The wall is not thickened.
- CBD is not dilated.

Spleen:-

Average size, homogenous.

Kidneys:-

- Both are average in size, shape and echogenicity
- No stones or dilated pelvicalyceal system
- No gross lesion

Para-aortic and pancreatic area: show no gross abnormalities.

No ascites

Urinary Bladder:-

- Average capacity with no stones or diverticular out pouching

Uterus and adenexal: no abnormalities could be detected

Opinion:

Un remarkable study

Signature Dr . Nermien Ramadan
PM 1:28 2024/07/08



Patient Name: Randa Salah Al Din Abbas Ibrahim		Date : 08/07/2024
Exam : X-RAY CHEST		Age : 48 Year(s)
Medical ID: 4194/2022		Pat. ID: 33125

Plain X-ray **Chest** (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*
- *Normal thoracic cage*
- *Left port-A-Cath in place*
 - Signature Dr . Nermien Ramadan
 - 08/07/2024 1:29 PM

جمهورية مصر العربية
بطاقة تحقيق الشخصية



راندا

صلاح الدين عباس ابراهيم

٦٤ زايد كومينكس - قري زايد

الشيخ زايد - الجيزة



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حاصلة على بكالوريوس خدمة اجتماعية

انثى مسلمة متزوجة

عبدالرحمن السيد احمد محمد

البطاقة سارية حتى ٢٠٢٧/٠١/٢٠



ID
٢٧٦.١٢٦.١٠.٤٤٤١

NAME: Randa salaheldeen
DATE: 29/7 /2026

Trans-vaginal ultrasound scanning revealed: -

○ **Uterus: -**

- AVF, average size uterus measuring about: 8x4x4cm in its longitudinal, transverse & AP diameters respectively.
- The myometrium shows small anterior wall fibroid measures 9x8mm
- The endometrial line is 11mm showing tiny cystic areas for follow up
- The cervix is of average size showing multiple Nabothian cysts

○ **Ovaries: -**

- Both ovaries are average size with no focal lesion
- No gross adnexal soft tissue masses or cysts.
- No free fluid in the Douglas pouch.

Best regards
Dr. NERMEEN RAMADAN FEBR, MD